SOMATIC SYMPTOM AND RELATED DISORDERS AND DISSOCIATIVE DISORDERS:

Understanding the Relationship Between the Mind, Body, Memory, and Identity

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Faculty



Learning Outcomes:

By the end of the lesson, students should be able to:

1. Differentiate somatic symptom and related disorders from dissociative disorders.
2. Describe the clinical features of somatic symptom disorder, illness anxiety disorder, conversion disorder, and psychological factors affecting medical conditions.
3. Explain depersonalization-derealization disorder, dissociative amnesia, and dissociative identity disorder.
4. Analyze biopsychosocial factors that may contribute to these conditions.
5. Apply appropriate diagnostic distinctions to brief clinical scenarios.
6. Identify evidence-informed approaches to treatment and ethical clinical practice.



Topic Outline:

PART A — SOMATIC SYMPTOM AND RELATED DISORDERS

- Somatic Symptom Disorder
- Illness Anxiety Disorder
- Psychological Factors Affecting Other Medical Conditions
- Conversion Disorder (Functional Neurological Symptom Disorder)

PART B — DISSOCIATIVE DISORDERS

- Depersonalization-Derealization Disorder
- Dissociative Amnesia
- Dissociative Identity Disorder

INTEGRATION

- Causes and contributing factors
- Treatment approaches
- Clinical case analysis



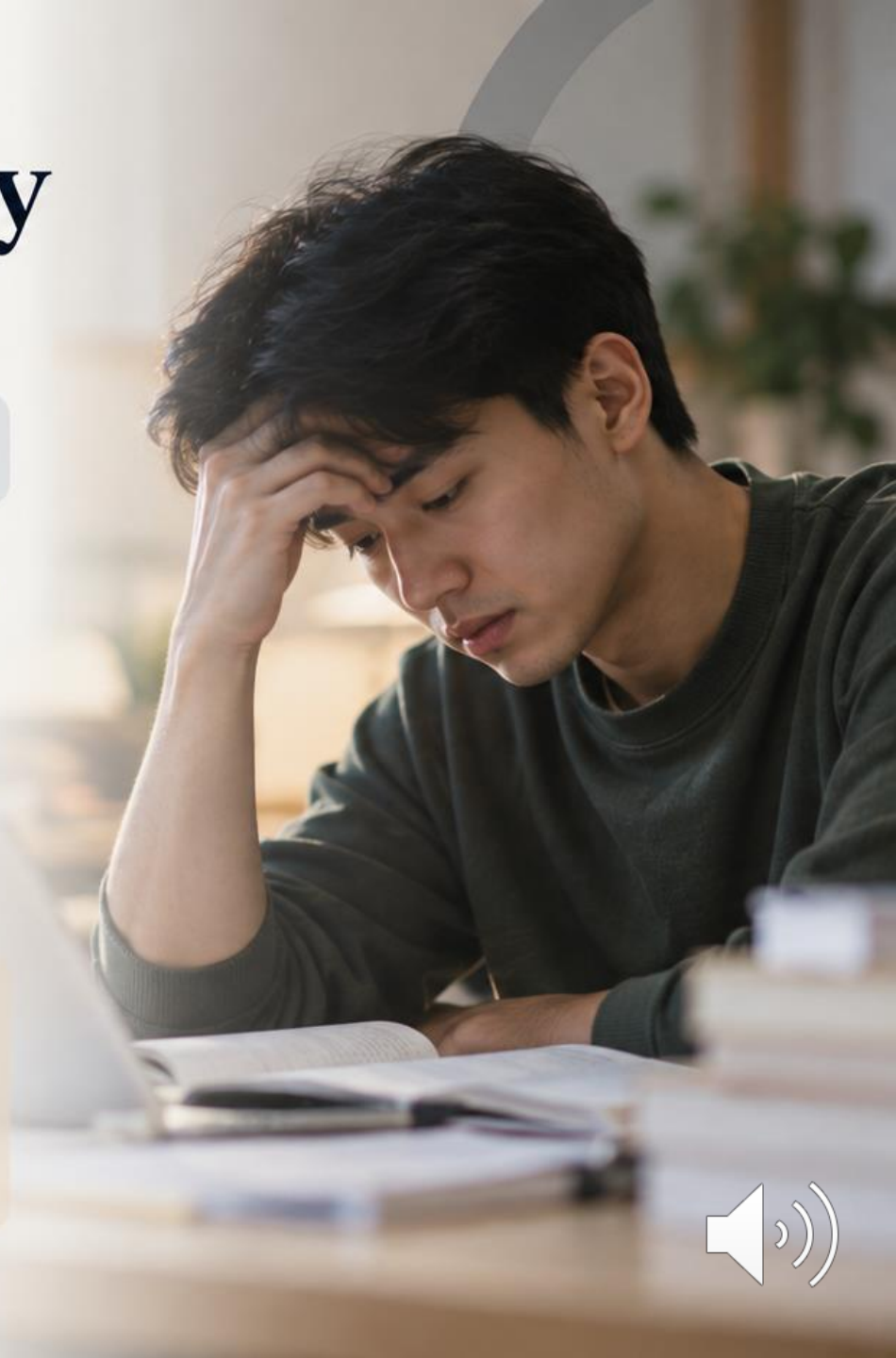
Icebreaker: The Mind–Body Connection

THINK – CHAT – SHARE

Imagine a student preparing for a major examination.

- Their stomach begins to hurt.
- Their heart beats faster.
- Their hands become sweaty.
- They feel dizzy despite having eaten.

Discussion question:
How might psychological stress contribute to these physical experiences?



SOMATIC SYMPTOM AND RELATED DISORDERS

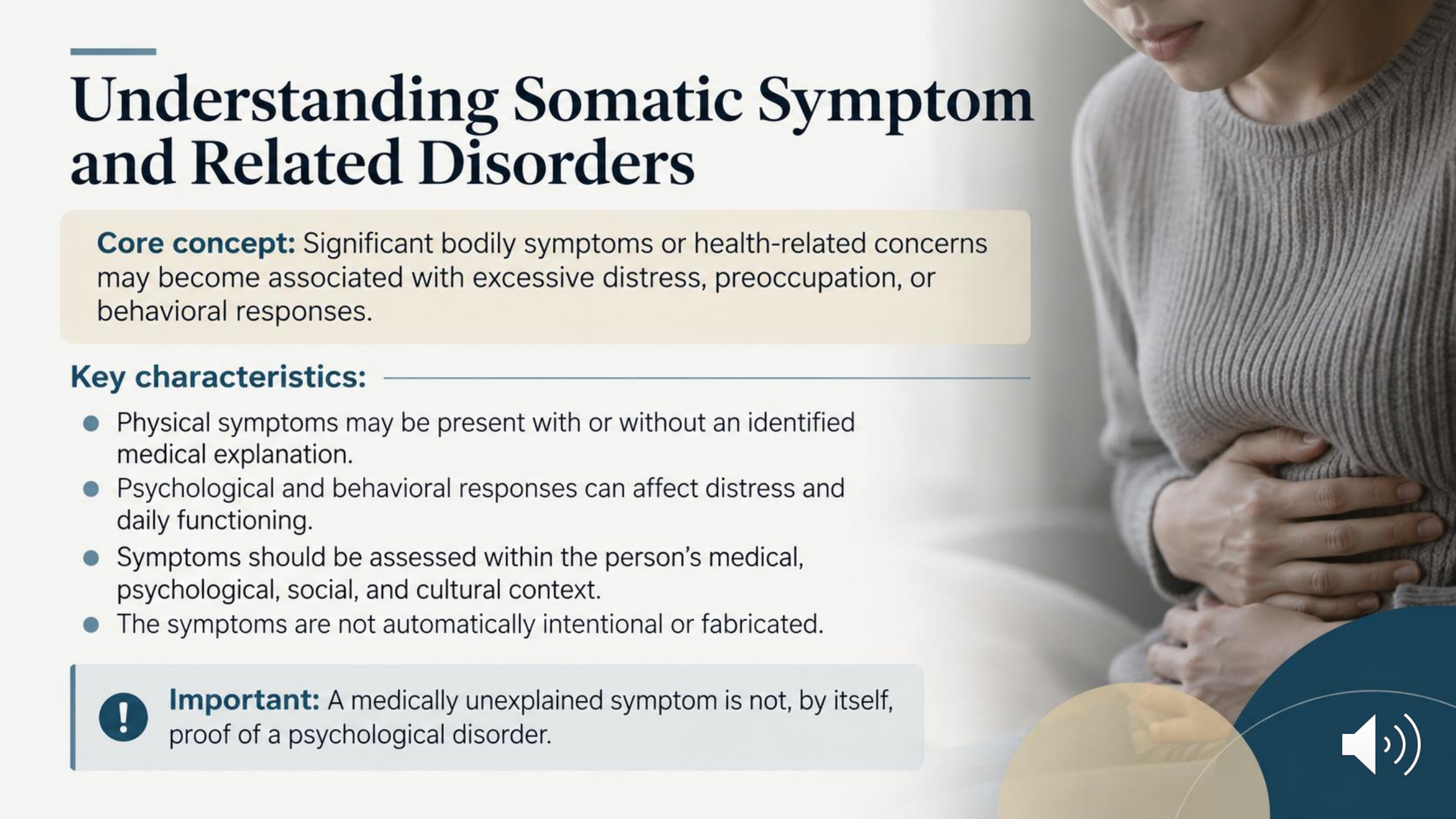
Understanding Somatic Symptom and Related Disorders

Core concept: Significant bodily symptoms or health-related concerns may become associated with excessive distress, preoccupation, or behavioral responses.

Key characteristics:

- Physical symptoms may be present with or without an identified medical explanation.
- Psychological and behavioral responses can affect distress and daily functioning.
- Symptoms should be assessed within the person's medical, psychological, social, and cultural context.
- The symptoms are not automatically intentional or fabricated.

Important: A medically unexplained symptom is not, by itself, proof of a psychological disorder.



Somatic Symptom Disorder

Clinical description

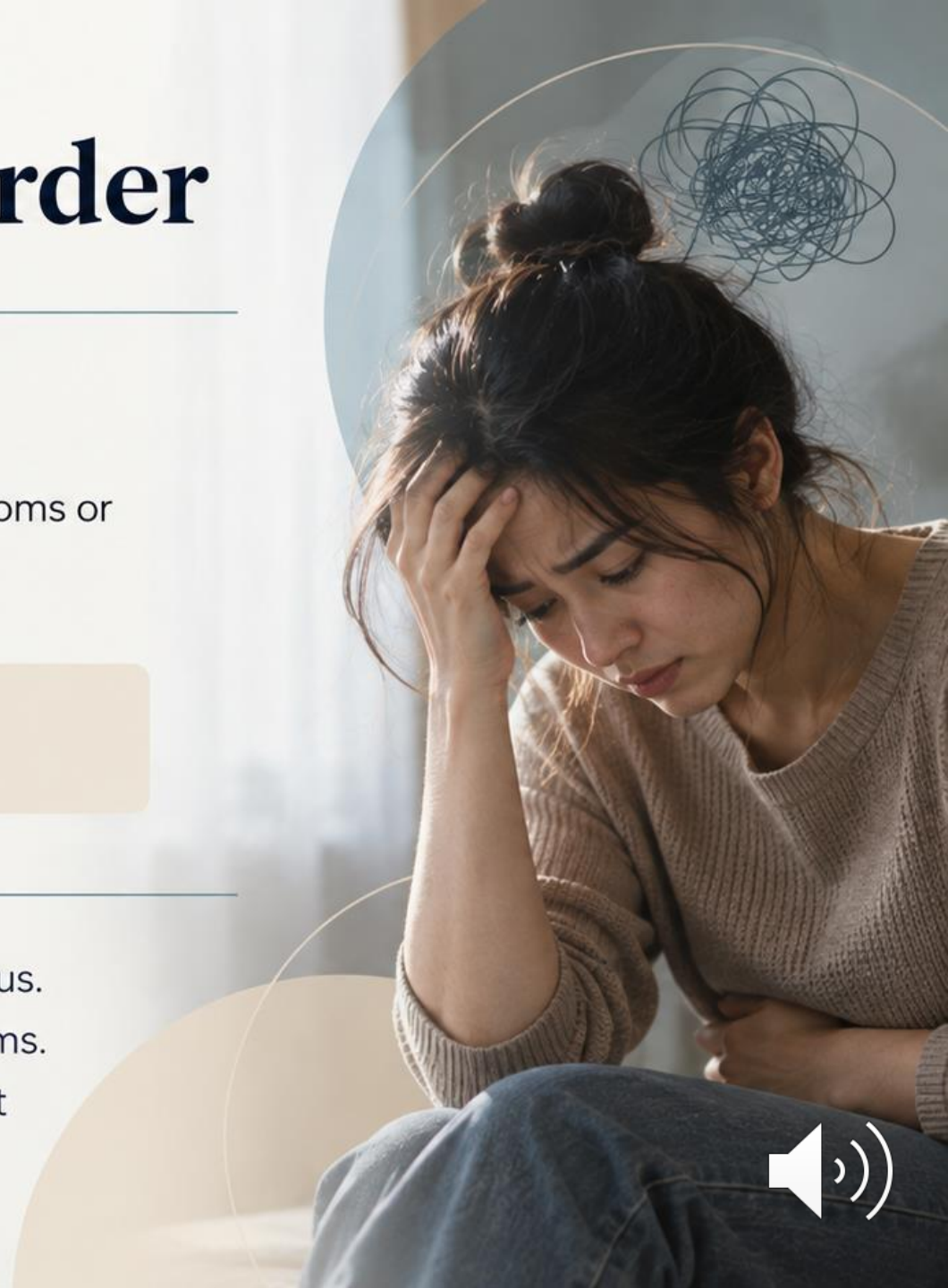
Somatic Symptom Disorder (SSD) involves:

- One or more distressing or disruptive bodily symptoms.
- Excessive thoughts, feelings, or behaviors related to the symptoms or associated health concerns.
- Significant distress or interference with daily life.

The presentation is typically persistent, often lasting **more than six months.**

Examples of excessive symptom-related responses

- Repeatedly interpreting ordinary bodily sensations as dangerous.
- Spending substantial time and energy worrying about symptoms.
- Frequently checking symptoms or seeking reassurance without lasting relief.



Somatic Symptom Disorder

Diagnostic Criteria | F45.1

A. One or more somatic symptoms that are distressing or result in significant disruption of daily life.

B. Excessive thoughts, feelings, or behaviors related to the somatic symptoms or associated health concerns as manifested by at least one of the following:

1. Disproportionate and persistent thoughts about the seriousness of one's symptoms.
2. Persistently high level of anxiety about health or symptoms.
3. Excessive time and energy devoted to these symptoms or health concerns.

C. Although any one somatic symptom may not be continuously present, the state of being symptomatic is persistent (typically more than 6 months).

Specify if:

With predominant pain (previously pain disorder): This specifier is for individuals whose somatic symptoms predominantly involve pain.

Specify if:

Persistent: A persistent course is characterized by severe symptoms, marked impairment, and long duration (more than 6 months).

Specify current severity:

Mild: Only one of the symptoms specified in Criterion B is fulfilled.
Moderate: Two or more of the symptoms specified in Criterion B are fulfilled.
Severe: Two or more of the symptoms specified in Criterion B are fulfilled, plus there are multiple somatic complaints (or one very severe somatic symptom).



Illness Anxiety Disorder

Clinical description

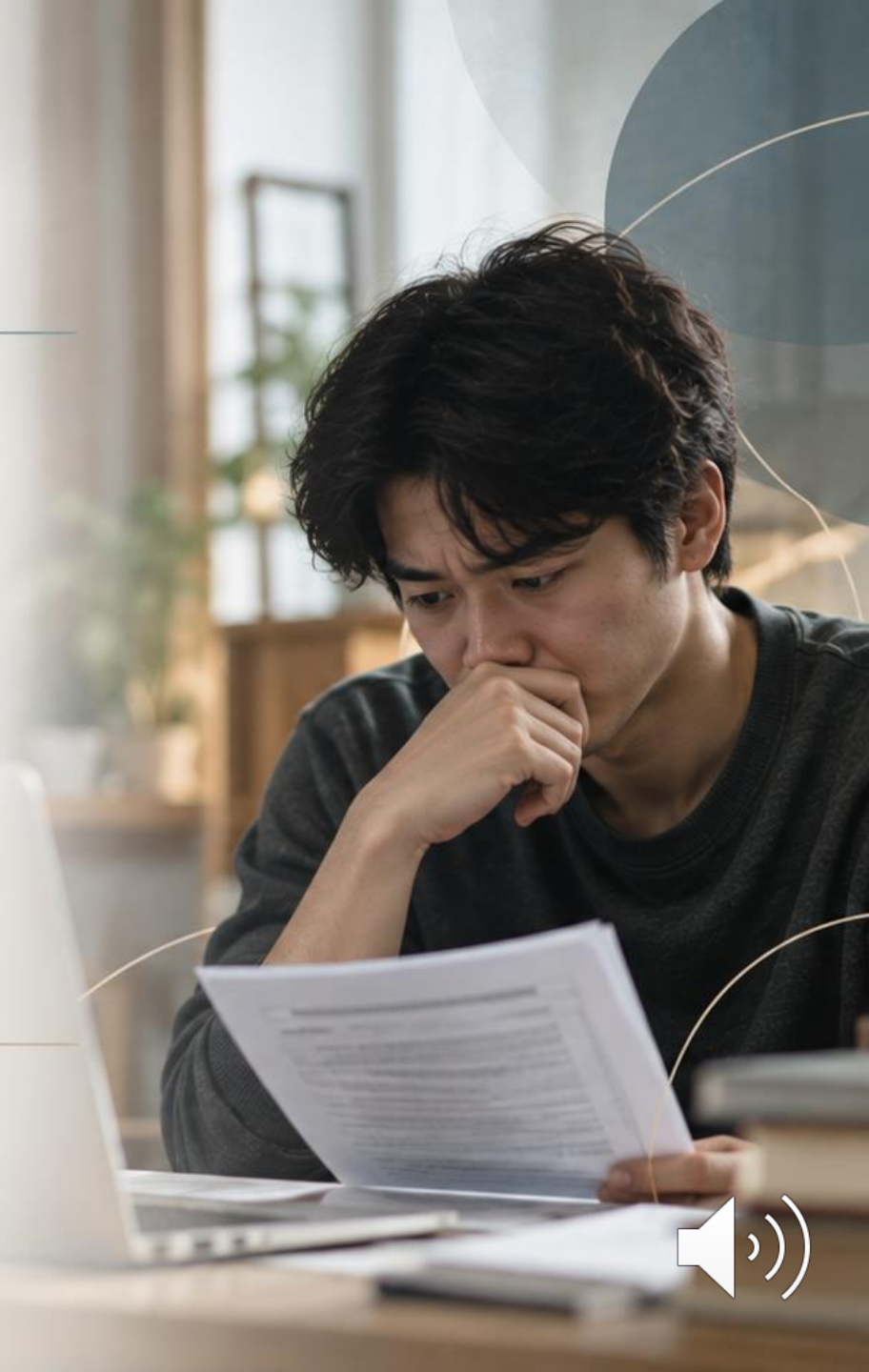
Illness Anxiety Disorder (IAD) involves **persistent preoccupation with having or acquiring a serious illness.**

Typical features:

- Somatic symptoms are absent or mild.
- Health anxiety is high.
- The person may frequently seek medical reassurance or avoid medical settings.
- The preoccupation generally persists for at least six months, although the feared illness may change.

Two possible behavioral patterns

- **Care-seeking:** Repeated appointments, examinations, or reassurance.
- **Care-avoidant:** Avoidance of doctors, hospitals, or feared medical information.



Illness Anxiety Disorder

Diagnostic Criteria | F45.21

A. Preoccupation with having or acquiring a serious illness.

B. Somatic symptoms are not present or, if present, are only mild in intensity. If another medical condition is present or there is a high risk for developing a medical condition (e.g., strong family history is present), the preoccupation is clearly excessive or disproportionate.

C. There is a high level of anxiety about health, and the individual is easily alarmed about personal health status.

D. The individual performs excessive health-related behaviors (e.g., repeatedly checks his or her body for signs of illness) or exhibits maladaptive avoidance (e.g., avoids doctor appointments and hospitals).

E. Illness preoccupation has been present for at least 6 months, but the specific illness that is feared may change over that period of time.

F. The illness-related preoccupation is not better explained by another mental disorder, such as somatic symptom disorder, panic disorder, generalized anxiety disorder, body dysmorphic disorder, obsessive-compulsive disorder, or delusional disorder, somatic type.

Specify whether:

Care-seeking type:	Medical care, including physician visits or undergoing tests and procedures, is frequently used.
Care-avoidant type:	Medical care is rarely used.



SSD vs. Illness Anxiety Disorder

Feature	Somatic Symptom Disorder	Illness Anxiety Disorder
Main concern	Distressing bodily symptoms and responses to them	Fear of having or acquiring serious illness
Somatic symptoms	One or more significant symptoms	Absent or relatively mild
Health-related anxiety	May be prominent	Central feature
Daily functioning	May be substantially impaired	May be impaired by illness preoccupation
Diagnosis	Requires the full diagnostic pattern	Requires the full diagnostic pattern

Remember: Neither diagnosis requires assuming that symptoms are fabricated.

Psychological Factors Affecting Other Medical Conditions

Core concept

Psychological or behavioral factors can adversely affect an existing medical condition.

Examples include:

- Stress that worsens the management of a medical illness.
- Persistent anxiety that interferes with necessary medical care.
- Difficulty adhering to a prescribed treatment plan because of psychological factors.
- Behaviors that increase health risks or aggravate a medical condition.

Key distinction:
A medical condition is present, and psychological or behavioral factors adversely affect its course, treatment, or outcome.



Psychological Factors Affecting Other Medical Conditions

Diagnostic Criteria | F54

A. A medical symptom or condition (other than a mental disorder) is present.

B. Psychological or behavioral factors adversely affect the medical condition in one or more ways:

1. The factors have influenced the course of the medical condition as shown by a close temporal association between the psychological factors and the development or exacerbation of, or delayed recovery from, the medical condition.
2. The factors interfere with the treatment of the medical condition (e.g., poor adherence).
3. The factors constitute additional well-established health risks for the individual.
4. The factors influence the underlying pathophysiology, precipitating or exacerbating symptoms or necessitating medical attention.

C. The psychological and behavioral factors in Criterion B are not better explained by another mental disorder (e.g., panic disorder, major depressive disorder, posttraumatic stress disorder).

Specify current severity

Mild:

Increases medical risk (e.g., inconsistent adherence with antihypertension treatment).

Moderate:

Aggravates underlying medical condition (e.g., anxiety aggravating asthma).

Severe:

Results in medical hospitalization or emergency room visit.

Extreme:

Results in severe, life-threatening risk (e.g., ignoring heart attack symptoms).

Conversion Disorder
(Functional Neurological Symptom Disorder)

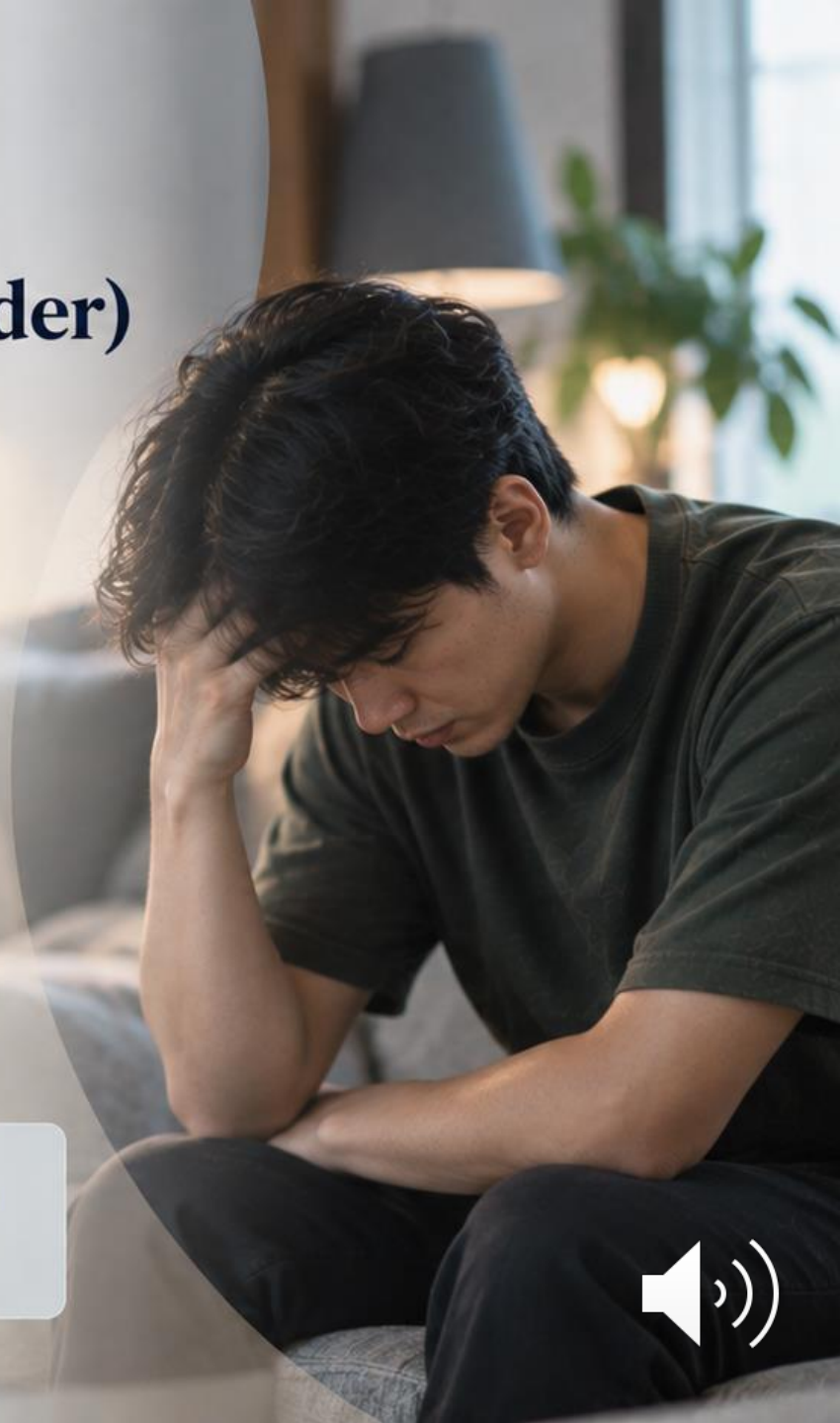
Clinical description

Conversion Disorder, also called Functional Neurological Symptom Disorder, involves neurological symptoms that are incompatible with recognized neurological disease patterns.

Possible symptoms:

- Weakness or paralysis.
- Abnormal movements or tremors.
- Changes in speech.
- Episodes resembling seizures.
- Sensory disturbances, such as altered vision or sensation.

Diagnostic principle: Positive clinical evidence of incompatibility with recognized neurological disease is important.



Conversion Disorder: Related Concepts and Mechanisms

Closely related concepts

- **Functional neurological symptoms:** Neurological symptoms associated with altered nervous-system functioning.
- **Factitious disorder:** Intentional falsification or induction of symptoms in association with the sick role.
- **Malingering:** Intentional production or exaggeration of symptoms for an external incentive; not itself a mental disorder.
- **Somatic symptom disorder:** Distressing bodily symptoms accompanied by excessive symptom-related thoughts, feelings, or behaviors.

Unconscious processes:
Psychological processes may be relevant, but unconscious conflict is not required to establish conversion disorder.



Functional Neurological Symptom Disorder
(Conversion Disorder)

Diagnostic Criteria

A. One or more symptoms of altered voluntary motor or sensory function.

B. Clinical findings provide evidence of incompatibility between the symptom and recognized neurological or medical conditions.

C. The symptom or deficit is not better explained by another medical or mental disorder.

D. The symptom or deficit causes clinically significant distress or impairment in social, occupational, or other important areas of functioning or warrants medical evaluation.

Coding note: The ICD-10-CM code depends on the symptom type.

Specify symptom type

F44.4	With weakness or paralysis
F44.4	With abnormal movement (e.g., tremor, dystonia, myoclonus, gait disorder)
F44.4	With swallowing symptoms
F44.4	With speech symptom (e.g., dysphonia, slurred speech)
F44.5	With attacks or seizures
F44.6	With anesthesia or sensory loss
F44.6	With special sensory symptom (e.g., visual, olfactory, or hearing disturbance)
F44.7	With mixed symptoms

Specify if

Acute episode: Symptoms present for less than 6 months.

Persistent: Symptoms occurring for 6 months or more.

Specify if

With psychological stressor: *(specify stressor)*

Without psychological stressor



SS and Related Disorders

- **Factitious Disorder -** A person deliberately falsifies, exaggerates, or induces physical or psychological symptoms to assume the sick role, even without an obvious external reward.

- **Other Specified Somatic Symptom and Related Disorder -** Somatic symptoms cause significant distress or impairment, but the presentation does not meet the full criteria for a specific disorder. The clinician identifies the specific reason why the criteria are not met.

- **Unspecified Somatic Symptom and Related Disorder -** Somatic symptoms cause significant distress or impairment, but there is insufficient information to determine the specific disorder or why the full criteria are not met.

Factitious Disorder

Factitious Disorder Imposed on Self | F68.10

A. Falsification of physical or psychological signs or symptoms, or induction of injury or disease, associated with identified deception.

B. The individual presents himself or herself to others as ill, impaired, or injured.

C. The deceptive behavior is evident even in the absence of obvious external rewards.

D. The behavior is not better explained by another mental disorder, such as delusional disorder or another psychotic disorder.

Specify:

- **Single episode**
- **Recurrent episodes** (two or more events of falsification of illness and/or induction of injury)

Factitious Disorder Imposed on Another (Previously Factitious Disorder by Proxy) | F68.A

A. Falsification of physical or psychological signs or symptoms, or induction of injury or disease, in another, associated with identified deception.

B. The individual presents another individual (victim) to others as ill, impaired, or injured.

C. The deceptive behavior is evident even in the absence of obvious external rewards.

D. The behavior is not better explained by another mental disorder, such as delusional disorder or another psychotic disorder.

Note: The perpetrator, not the victim, receives this diagnosis.

Specify:

- **Single episode**
- **Recurrent episodes** (two or more events of falsification of illness and/or induction of injury)

Other Specified Somatic Symptom and Related Disorder

F45.8

This category applies to presentations in which symptoms characteristic of a somatic symptom and related disorder that cause clinically significant distress or impairment in social, occupational, or other important areas of functioning predominate but do not meet the full criteria for any of the disorders in the somatic symptom and related disorders diagnostic class.

Examples

Examples of presentations that can be specified using the "other specified" designation include the following:

1. **Brief somatic symptom disorder:** Duration of symptoms is less than 6 months.
2. **Brief illness anxiety disorder:** Duration of symptoms is less than 6 months.
3. **Illness anxiety disorder without excessive health-related behaviors or maladaptive avoidance:** Criterion D for illness anxiety disorder is not met.
4. **Pseudocyesis:** A false belief of being pregnant that is associated with objective signs and reported symptoms of pregnancy.

Unspecified Somatic Symptom and Related Disorder

F45.9

This category applies to presentations in which symptoms characteristic of a somatic symptom and related disorder that cause clinically significant distress or impairment in social, occupational, or other important areas of functioning predominate but do not meet the full criteria for any of the disorders in the somatic symptom and related disorders diagnostic class. The unspecified somatic symptom and related disorder category should not be used unless there are decidedly unusual situations where there is insufficient information to make a more specific diagnosis.



Somatic Symptom and Related Disorders

1 Statistics

- **Somatic Symptom Disorder:** Approximately **5–7%** in some community studies.
- **Illness Anxiety Disorder:** Estimated at around **1–2%** in some populations.
- **Conversion Disorder:** Some studies report **5–10 cases** per 100,000 people annually.
- **Factitious Disorder:** Exact prevalence is unknown.

2 Contributing Factors

- **Biological:** Genetic vulnerability and heightened sensitivity to bodily sensations.
- **Psychological:** Anxiety, depression, and excessive worry about physical symptoms.
- **Environmental:** Trauma, childhood adversity, and stressful life events.
- **Social:** Family conflict, work or academic pressure, and learned illness behaviors.

3 Treatment

- **Cognitive Behavioral Therapy (CBT):** Changes unhelpful thoughts and behaviors related to symptoms.
- **Medical follow-up:** Provides consistent care and monitors physical symptoms.
- **Stress management:** Uses relaxation and coping strategies.
- **Medication:** Antidepressants may help when anxiety or depression is present.
- **Behavioral interventions:** Encourage gradual return to normal daily activities.



DISSOCIATIVE DISORDERS

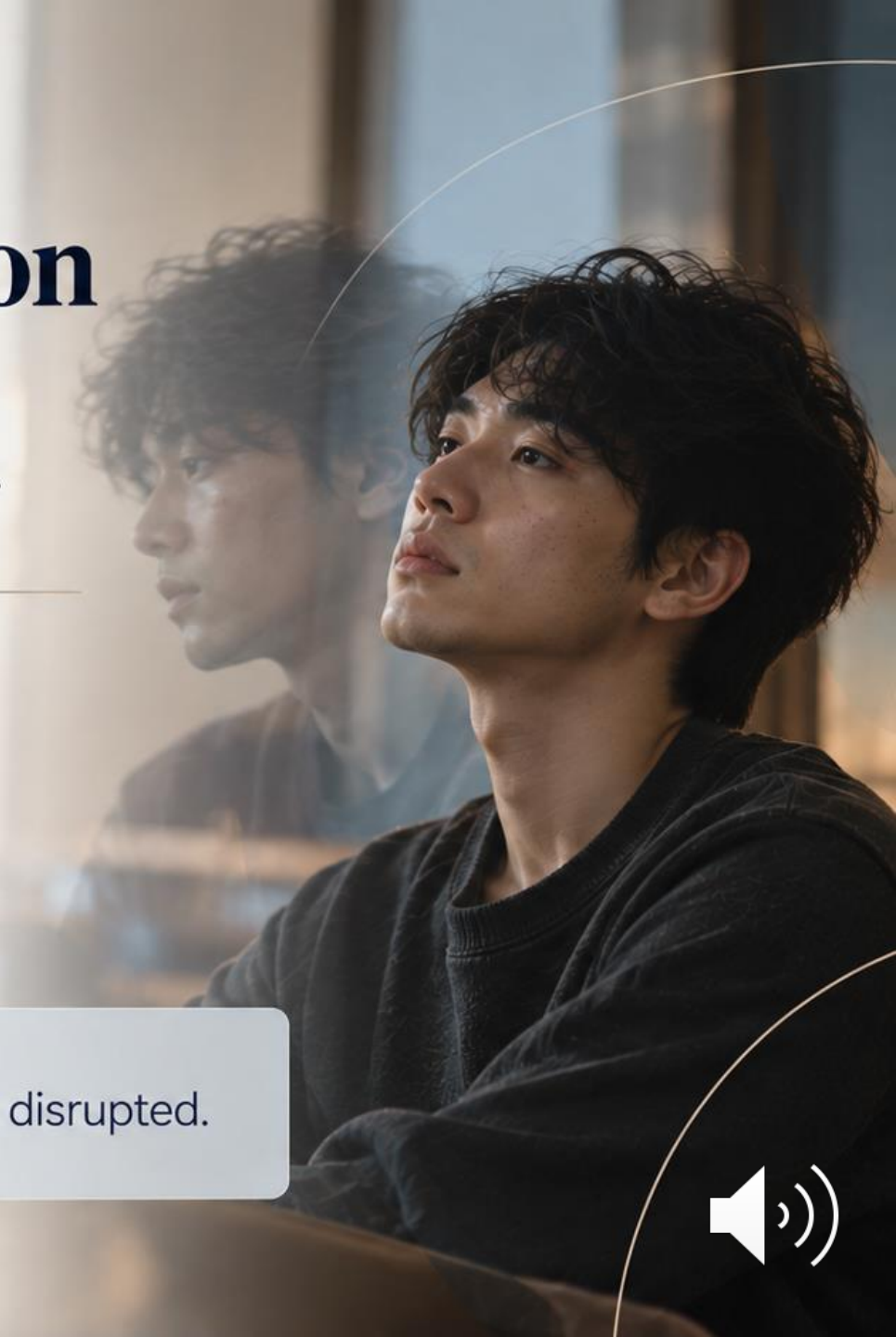
Understanding Dissociation

Dissociation refers to a disruption in the integration of aspects of consciousness, memory, identity, emotion, perception, or behavior.

Dissociative disorders may involve:

- Feeling detached from oneself.
- Experiencing the world as unreal.
- Gaps in autobiographical memory.
- Disruptions in identity and a sense of agency.

Core idea: Ordinary integration of experience may be disrupted.



Depersonalization-Derealization Disorder

Depersonalization

Feeling detached from oneself, one's body, thoughts, emotions, or actions.

> **Example:** "It feels as if I am observing myself from outside."

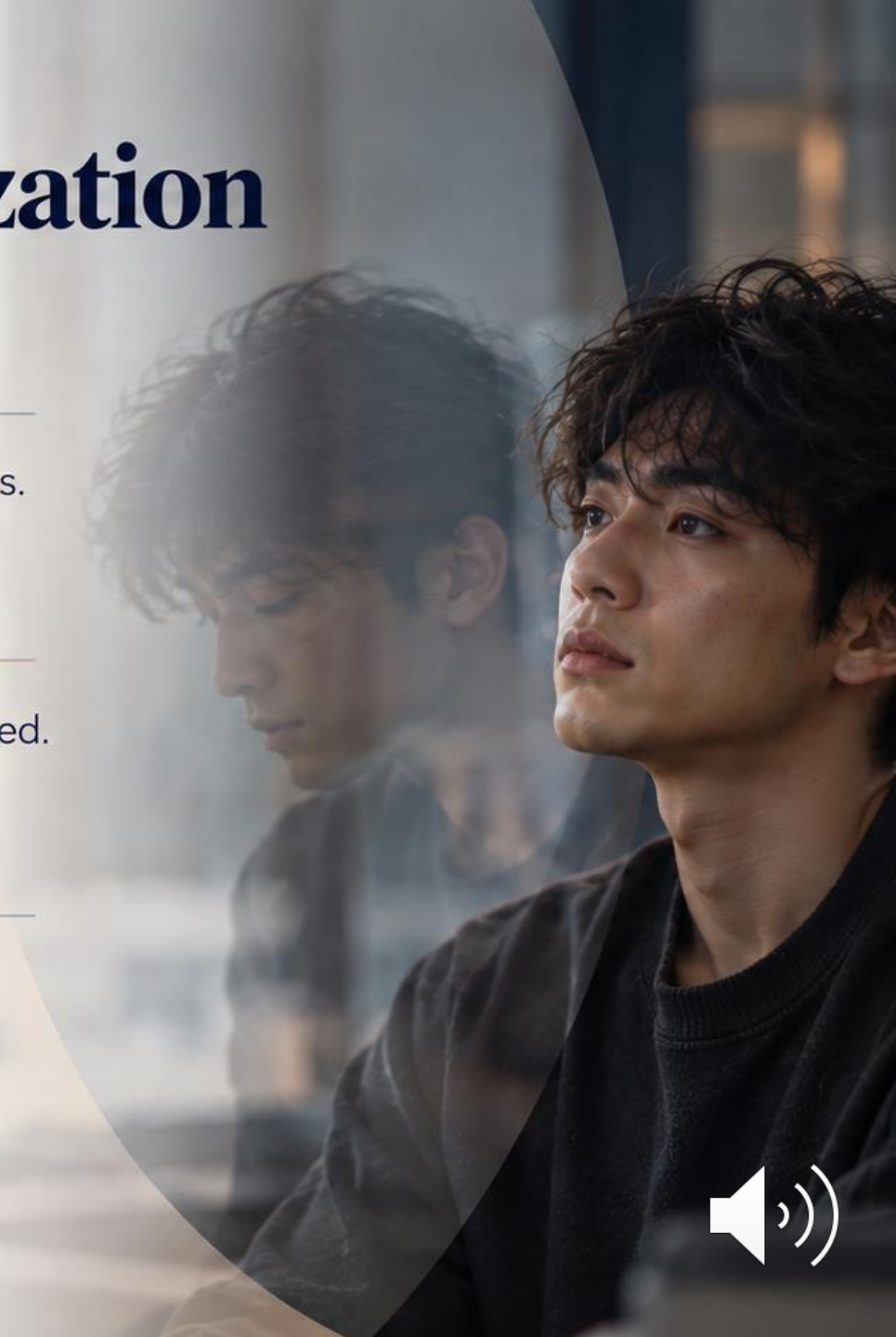
Derealization

Feeling that the external world is unreal, dreamlike, distant, or visually altered.

> **Example:** "My surroundings look artificial, as though I am in a movie."

Key features

- Experiences are persistent or recurrent.
- They cause significant distress or impairment.
- Reality testing remains intact during the experiences.
- Other medical, substance-related, and psychiatric explanations must be considered.



Depersonalization/Derealization Disorder

Diagnostic Criteria | F48.1

A. The presence of persistent or recurrent experiences of depersonalization, derealization, or both:

1. **Depersonalization:** Experiences of unreality, detachment, or being an outside observer with respect to one's thoughts, feelings, sensations, body, or actions (e.g., perceptual alterations, distorted sense of time, unreal or absent self, emotional and/or physical numbing).
2. **Derealization:** Experiences of unreality or detachment with respect to surroundings (e.g., individuals or objects are experienced as unreal, dreamlike, foggy, lifeless, or visually distorted).

B. During the depersonalization or derealization experiences, reality testing remains intact.

C. The symptoms cause clinically significant distress or impairment in social, occupational, or other important areas of functioning.

D. The disturbance is not attributable to the physiological effects of a substance (e.g., a drug of abuse, medication) or another medical condition (e.g., seizures).

E. The disturbance is not better explained by another mental disorder, such as schizophrenia, panic disorder, major depressive disorder, acute stress disorder, posttraumatic stress disorder, or another dissociative disorder.

Dissociative Amnesia

Clinical description

Dissociative amnesia involves an inability to recall important autobiographical information, usually related to stressful or traumatic experiences, that is inconsistent with ordinary forgetting.

Possible patterns

- **Localized:** Inability to recall events from a specific period.
- **Selective:** Recalling some, but not all, events from a period.
- **Generalized:** Extensive loss of autobiographical memory; uncommon.

Important exclusions

Memory difficulties must not be better explained by ordinary forgetting, neurological illness, substance effects, or another condition.



Dissociative Amnesia

Diagnostic Criteria | F44.0

A. An inability to recall important autobiographical information, usually of a traumatic or stressful nature, that is inconsistent with ordinary forgetting.

Note: Dissociative amnesia most often consists of localized or selective amnesia for a specific event or events; or generalized amnesia for identity and life history.

B. The symptoms cause clinically significant distress or impairment in social, occupational, or other important areas of functioning.

C. The disturbance is not attributable to the physiological effects of a substance (e.g., alcohol or other drug of abuse, a medication) or a neurological or other medical condition (e.g., partial complex seizures, transient global amnesia, sequelae of a closed head injury/traumatic brain injury, other neurological condition).

D. The disturbance is not better explained by dissociative identity disorder, posttraumatic stress disorder, acute stress disorder, somatic symptom disorder, or major or mild neurocognitive disorder.

Coding note: The code for dissociative amnesia without dissociative fugue is **F44.0.** The code for dissociative amnesia with dissociative fugue is **F44.1.**

Specify if:

F44.1 With dissociative fugue: Apparently purposeful travel or bewildered wandering that is associated with amnesia for identity or for other important autobiographical information.

Dissociative Identity Disorder (DID)

Clinical description

Dissociative Identity Disorder involves:

- Disruption of identity characterized by **two or more distinct personality states**, or an experience of possession in some cultural presentations.
- Marked **discontinuity in the sense of self and agency**.
- **Related changes** in affect, behavior, consciousness, memory, perception, cognition, or sensory-motor functioning.
- Recurrent **gaps in recall** of everyday events, important personal information, or traumatic events.
- Clinically significant **distress or impairment**.

Assessment must consider **cultural context** and **alternative explanations**.



DID: Characteristics, Suggestibility, and the Question of Faking

Clinical characteristics

- Disruptions in identity and sense of agency.
- Gaps in memory that extend beyond ordinary forgetting.
- Changes in self-experience and behavior.
- Possible co-occurring trauma-related symptoms, anxiety, depression, or other difficulties.

Can DID be faked?

- Symptoms can be intentionally imitated, as can symptoms of many other conditions.
- Suggestibility and expectations may influence symptom reports and interpretations.
- Symptom imitation does not establish that every case is fabricated.
- Diagnosis requires comprehensive, neutral assessment.

Avoid assumptions based on media portrayals.



Dissociative Identity Disorder

Diagnostic Criteria F44.81

A. Disruption of identity characterized by two or more distinct personality states, which may be described in some cultures as an experience of possession. The disruption in identity involves marked discontinuity in sense of self and sense of agency, accompanied by related alterations in affect, behavior, consciousness, memory, perception, cognition, and/or sensory-motor functioning. These signs and symptoms may be observed by others or reported by the individual.

B. Recurrent gaps in the recall of everyday events, important personal information, and/or traumatic events that are inconsistent with ordinary forgetting.

C. The symptoms cause clinically significant distress or impairment in social, occupational, or other important areas of functioning.

D. The disturbance is not a normal part of a broadly accepted cultural or religious practice.

Note: In children, the symptoms are not better explained by imaginary playmates or other fantasy play.

E. The symptoms are not attributable to the physiological effects of a substance (e.g., blackouts or chaotic behavior during alcohol intoxication) or another medical condition (e.g., complex partial seizures).



Other Specified Dissociative Disorder

Diagnostic Criteria F44.89

This category applies to presentations in which symptoms characteristic of a dissociative disorder that cause clinically significant distress or impairment in social, occupational, or other important areas of functioning predominate but do not meet the full criteria for any of the disorders in the dissociative disorders diagnostic class. The other specified dissociative disorder category is used in situations in which the clinician chooses to communicate the specific reason that the presentation does not meet the criteria for any specific dissociative disorder. This is done by recording "other specified dissociative disorder" followed by the specific reason (e.g., "dissociative trance").

Examples of presentations that can be specified using the "other specified" designation include the following:

1. **Chronic and recurrent syndromes of mixed dissociative symptoms:** This category includes identity disturbance associated with less-than-marked discontinuities in sense of self and agency, or alterations of identity or episodes of possession in an individual who reports no dissociative amnesia.
2. **Identity disturbance due to prolonged and intense coercive persuasion:** Individuals who have been subjected to intense coercive persuasion (e.g., brainwashing, thought reform, indoctrination while captive, torture, long-term political imprisonment, recruitment by sects/cults or by terror organizations) may present with prolonged changes in, or conscious questioning of, their identity.
3. **Acute dissociative reactions to stressful events:** This category is for acute, transient conditions that typically last less than 1 month, and sometimes only a few hours or days. These conditions are characterized by constriction of consciousness; depersonalization; derealization; perceptual disturbances (e.g., time slowing, macropsia); microamnesias; transient stupor; and/or alterations in sensory-motor functioning (e.g., analgesia, paralysis).
4. **Dissociative trance:** This condition is characterized by an acute narrowing or complete loss of awareness of immediate surroundings that manifests as profound unresponsiveness or insensitivity to environmental stimuli. The unresponsiveness may be accompanied by minor stereotyped behaviors (e.g., finger movements) of which the individual is unaware and/or that he or she cannot control, as well as transient paralysis or loss of consciousness. The dissociative trance is not a normal part of a broadly accepted collective cultural or religious practice.

Unspecified Dissociative Disorder

F44.9

This category applies to presentations in which symptoms characteristic of a dissociative disorder that cause clinically significant distress or impairment in social, occupational, or other important areas of functioning predominate but do not meet the full criteria for any of the disorders in the dissociative disorders diagnostic class. The unspecified dissociative disorder category is used in situations in which the clinician chooses *not* to specify the reason that the criteria are not met for a specific dissociative disorder and includes presentations for which there is insufficient information to make a more specific diagnosis (e.g., in emergency room settings).

Statistics on Dissociative Disorders

Dissociative disorder / finding	Reported statistics
Dissociative disorders overall	Some studies estimate prevalence at approximately **1–5%** of the population.
Dissociative Identity Disorder (DID)	Approximately **1–1.5%** in some population estimates.
Dissociative Amnesia	One community study reported a **1.8%** prevalence within the past year.
Depersonalization/Derealization Disorder	Approximately **1%** of the general population, with systematic-review estimates ranging from **0% to 1.9%**.
Transient dissociative experiences	Temporary depersonalization or derealization experiences have been reported by **26–74%** of people across studies; these experiences do not necessarily indicate a disorder.

Important distinction: Experiencing occasional dissociation is not the same as having a dissociative disorder. Diagnosis requires clinically significant symptoms and appropriate assessment.

Biological Contributions, Trauma, and Memory

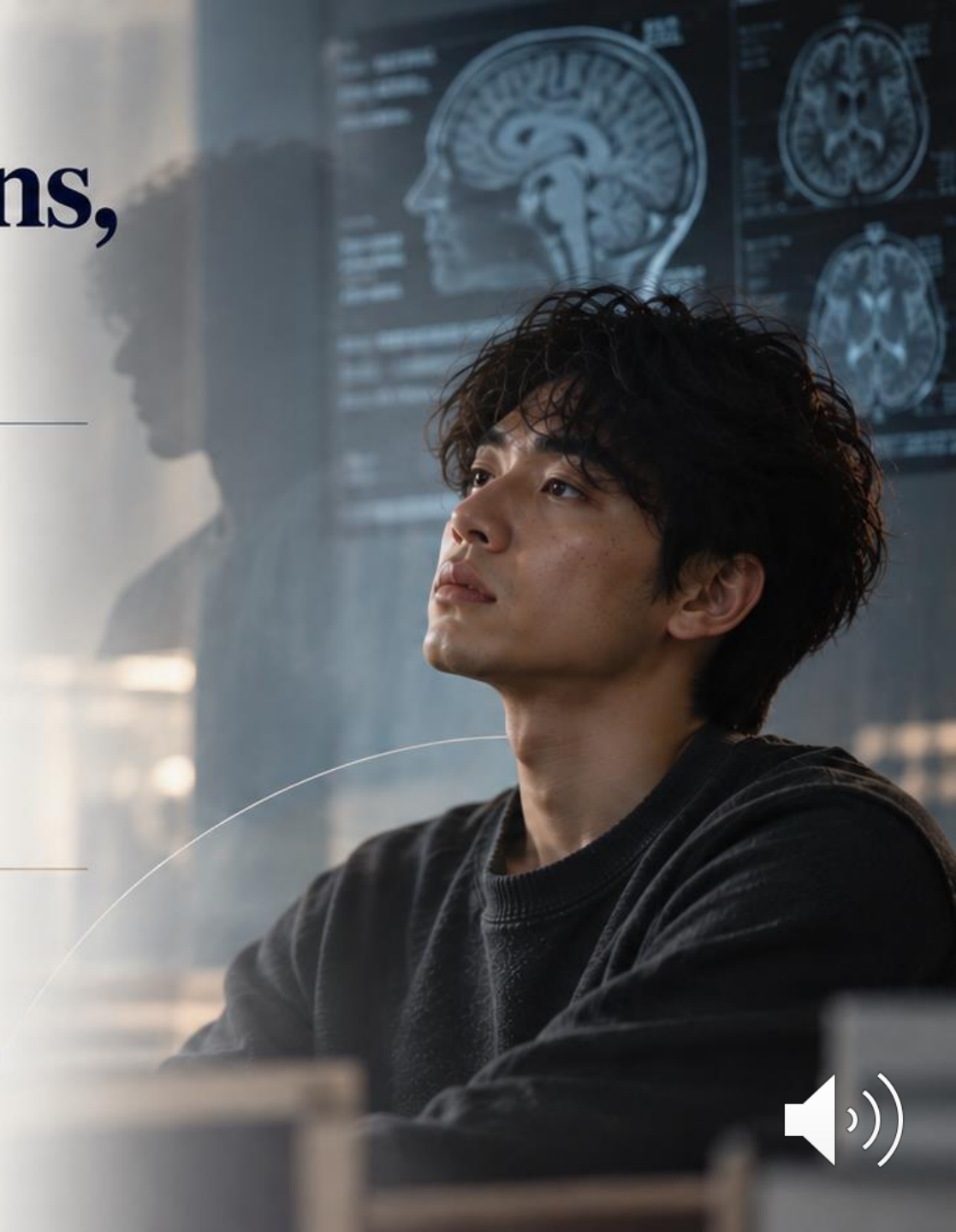
An integrative view of dissociation

Potentially relevant factors include:

- **Psychological:** Stress, coping processes, and the effects of overwhelming experiences.
- **Biological:** Individual differences in brain functioning and memory-related processes.
- **Social and cultural:** Social context, cultural interpretations, and the language used to describe experiences.

Real memories and false memories

- Human memory is reconstructive, not a perfect recording.
- Confidence does not guarantee accuracy.
- Suggestion and repeated questioning can influence recollection.
- Memories should not be treated as verified facts without appropriate corroboration.



Treatment and Clinical Management

Condition	General treatment principles
Somatic Symptom Disorder	Collaborative care, psychoeducation, psychological treatment, and coordinated medical follow-up.
Illness Anxiety Disorder	Cognitive behavioral approaches targeting health anxiety, checking, avoidance, and reassurance-seeking.
Conversion Disorder	Clear explanation of the diagnosis, rehabilitation when indicated, and psychological treatment tailored to the person's needs.
Depersonalization-Derealization Disorder	Treatment of contributing or co-occurring conditions and psychological approaches targeting distress.
Dissociative Amnesia	Safety, careful assessment, supportive treatment, and management of associated difficulties.
Dissociative Identity Disorder	Individualized, phase-oriented psychotherapy focused on safety, stabilization, and improved functioning.



Virtual Case Analysis: What Is the Best Explanation?

CASE A — Persistent Bodily Symptoms

Mara experiences frequent abdominal pain. Medical evaluation identifies no clear explanation for all her symptoms. She spends several hours daily worrying about the pain, repeatedly checks her body, and struggles to attend classes.

CASE B — Fear of Serious Illness

Daniel experiences occasional mild headaches. He persistently fears having a serious neurological disease, searches for symptoms online, and repeatedly seeks reassurance.

CASE C — Feeling Detached

Lea repeatedly feels as though she is observing herself from outside her body. She knows the experience is subjective but finds it frightening and disruptive.

CASE D — Memory Gaps

Paolo experiences significant gaps in autobiographical memory that cannot be explained by ordinary forgetting. A comprehensive assessment is needed to determine the cause.

Discussion task: Identify the most relevant diagnostic possibility and explain your reasoning.

Knowledge Check

Choose the best answer for each item.



1 **A person has significant bodily symptoms and spends much of the day worrying about them. Which diagnosis is most relevant to consider?**

A. Dissociative amnesia
B. Somatic Symptom Disorder
C. Depersonalization-Derealization Disorder
D. Dissociative Identity Disorder

2 **A person fears having a serious illness despite experiencing only mild bodily symptoms. Which diagnosis is most relevant to consider?**

A. Illness Anxiety Disorder
B. Conversion Disorder
C. Dissociative Identity Disorder
D. Dissociative Amnesia

3 **A patient experiences weakness with clinical findings that are incompatible with recognized neurological disease patterns. Which condition should be considered?**

A. Illness Anxiety Disorder
B. Dissociative Amnesia
C. Functional Neurological Symptom Disorder
D. Depersonalization-Derealization Disorder

4 **A person feels detached from their own thoughts and body but recognizes that the experience is subjective. Which symptom best describes this experience?**

A. Derealization
B. Depersonalization
C. Dissociative amnesia
D. Illness anxiety

5 **Which statement about dissociative identity disorder is most appropriate?**

A. It is simply the experience of having different moods.
B. It can be diagnosed whenever someone reports a memory gap.
C. It requires careful assessment of identity disruption, memory, functioning, and alternative explanations.
D. It is always intentionally fabricated.

Synthesis: What Should We Remember?

Somatic symptom and related disorders

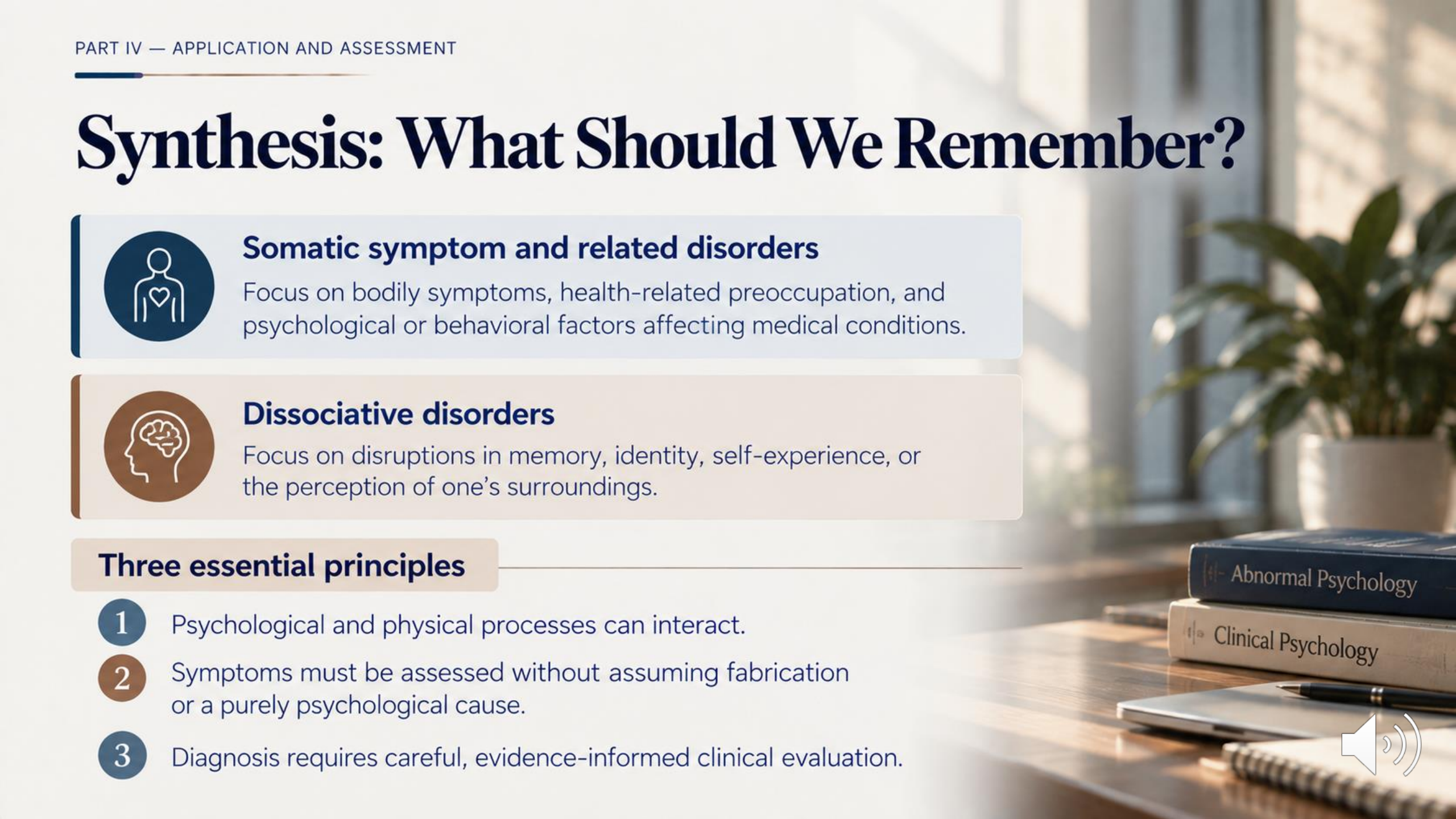
Focus on bodily symptoms, health-related preoccupation, and psychological or behavioral factors affecting medical conditions.

Dissociative disorders

Focus on disruptions in memory, identity, self-experience, or the perception of one's surroundings.

Three essential principles

1. Psychological and physical processes can interact.
2. Symptoms must be assessed without assuming fabrication or a purely psychological cause.
3. Diagnosis requires careful, evidence-informed clinical evaluation.



Homework: Clinical Reflection

Individual Assignment: Mind, Body, Memory, and Identity

Write a 300–400-word reflection addressing the following:

1. Choose one disorder discussed in class.
2. Explain its defining clinical features in your own words.
3. Describe how biological, psychological, and social factors may contribute to the person's experience.
4. Identify one common misconception about the disorder and correct it using a credible academic source.
5. Explain why ethical and culturally sensitive assessment is important.



Submission:
Next class meeting

Format:
PDF or Word document

Requirement:
Include at least two credible references in APA 7th edition format.

References

APA 7th Edition

American Psychiatric Association. (2022). *Diagnostic and statistical manual of mental disorders* (5th ed., text rev.). https://doi.org/10.1176/appi.books.9780890425787

Barlow, D. H., & Durand, V. M. (2015). *Abnormal psychology: An integrative approach* (7th ed.). Cengage Learning.

American Psychological Association. (n.d.). *Somatic symptom disorder*. *APA Dictionary of Psychology*. https://dictionary.apa.org/somatic-symptom-disorder

